Artwork Document Information

Product : T PROCHLOR TAB 5MG [FRONT]
SAP No. :
Date : 25 August 2025

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Remarks

1. Font : Adobe Garamond Pro Regular (Spacing -11.52pt / Size - 9.6pt)
2. Dimension : 197mm(w) x 280mm(h)

Colour

Black

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PROCHLOR TABLET 5MG

DESCRIPTION

A 6.5 mm diameter, round, convex scored, marked "UP" pink tablet. Containing 5mg prochlorperazine maleate per tablet.

INDICATIONS

Prochlor is indicated in the management of psychotic conditions including the treatment of schizophrenia and for production of quieting effect in hyperactive or excited psychotic patients. Prochlor can also be used to control severe nausea and vomiting including that associated with migraine or drug induced emesis and for the short-term symptomatic relief or vertigo as occurs in Meniere's disease or labyrinthitis.

PHARMACODYNAMICS

Prochlorperazine is a piperazine type of phenothiazine. Its antipsychotic effect may be due to the blockage of postsynaptic mesolimbic dopaminergic receptors in the brain, which also increases prolactin release by the pituitary. An alpha adrenergic blocking effect is also produced which depresses the release of hypothalamic and hypophyseal hormones.
Prochlorperazine acts centrally to inhibit or block the dopamine (D2) receptors in the medulary chemoreceptor trigger zone (CTZ) and peripherally by blocking the vagus nerve in the gastrointestinal tract. Its antiemetic effect may be augmented by their anticholinergic, sedative and antihistaminic effects.

PHARMACOKINETICS

Prochlorperazine is well absorbed from the GI tract but is subject to considerable first pass metabolism from the gut wall. It is also extensively metabolised in the liver and is excreted in the urine and bile. Plasma concentrations following oral administration are much lower than those following intramuscular injection, and are subject to wide inter-subject variation. There is no simple correlation between plasma concentrations of prochlorperazine and its metabolites, and therapeutic effect.
Prochlorperazine may be metabolised by hydroxylation and conjugation with glucuronic acid, N-oxidation, oxidation of the sulfur atom and dealkylation. Plasma half-life is reported to be only a few hours but elimination of the metabolites may be very prolonged. Prochlorperazine is extensively bound to plasma proteins, widely distributed in the body (it crosses the blood/brain barrier) and its metabolites cross the placental barrier and are excreted in milk. The rate of metabolism and excretion decreases in old age.

DOSAGE

Adult and adolescents:
Psychotic disorders
5 to l0mg (base) three to four times daily, the dosage being increased every two or three days as needed and tolerated.
Anxiety
5mg (base) three to four times daily, up to 20mg daily, for no longer than 12 weeks
Nausea and vomiting
5 to l0mg (base) three to four times daily, up to 40mg a day. Doses over 40mg per day should be limited to resistant cases.
Usual adult prescribing limits: Up to 150mg (base) daily for treatment of psychotic disorders.
Geriatric, emaciated or debilitated patients usually require a lower dose gradually increased as needed and tolerated.
Pediatric patients should use the oral syrup dosage form.
Route of Administration: Oral

OVERDOSAGE

Treatment is essentially symptomatic and supportive. The following may be considered:
Attempting early gastric lavage, avoid induction of vomiting. Activated charcoal slurry or saline cathartic may be administered. Always maintain respiratory function and body temperature of overdose patient, monitoring cardiovascular function for not less than 5 days. Cardiac arrhythmias can be controlled using intravenous phenytoin, 9-11mg per kg of body weight. Digitalize patients if necessary for cardiac failure. Vasopressors are recommended for hypotension, the use of norepinephrine or phenylphrine may be considered as these do not cause paradoxical hypotension. Convulsions should be controlled with diazepam followed by phenytoin, l5mg/kg while monitoring ECG. Benztropine or diphenhydramine can be administered to manage acute parkinson-like symptoms that may occur. Dialysis of phenothiazines have not been successful.
Up-to-date information on the treatment of overdose can be obtained from The National Poison Centre, Universiti Sains Malaysia.

TOXICOLOGY

Patients sensitive to one prochlorperazine may be sensitive to other phenothiazines.
Phenothiazines are not recommended for use during pregnancy

SIDE EFFECTS

More frequently, anticholinergic effects such as constipation, decreased sweating, dizziness, drowsiness, dry mouth and nasal congestion have been reported. Also, akathisia (restlessness or the need to keep moving), blurred vision, dystonic extrapyramidal effects (muscle spasms of face, neck and back, twitching and twisting movements), parkinsonian extrapyramidal effects, hypotension, pigmentary retinopathy and tardive dyskinesia may be experienced. Less frequently, difficulty in urinating, increased sensitivity of skin to sun, skin rash associated with contact dermatitis or other alergic reaction, or cholestatic jaundice have been seen. Changes in menstrual period, decreased sexual ability, unusual secretion of milk, swelling or pain in breasts, and unusual weight gain have been reported. Rarely, agranulocytosis, cholestatic jaundice, heat stroke especially in places of high heat and humidity, neuroleptic malignant syndrome [NMS], priapism and melanosis have been seen.

CONTRAINDICATIONS

Prochlorperazine is contraindicated in patients with severe cardiovascular disease, severe CNS depression or comatose states.

DRUG INTERACTIONS

Use of phenothiazines with alcohol or CNS depression-producing medication may result in increased CNS and respiratory depression and increased hypotensive effects. Concurrent use with amantadine, antidyskine-tics, antihistamines or

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anticholinergics may intensify anticholinergic side effects, as with tricyclic antidepressants, maprotiline or monoamine oxidases including procarbazine and selegiline where sedative effects may also be intensified. Phenothiazines lower the seizure threshold; concomittant use may necessitate dosage adjustment of anticonvulsants. Phenytoin metabolism may also be inhibited and may lead to toxicity. The risk of agranulocytosis is increased when used with antithyroid agents. Concurrent use of beta blockers, possibly including ophthalmics with phenothiazines may result in an increased plasma concentration of each medication which may result in increased effects and side effects. Phenothiazines may further increase the severity when used with other medicines which produces extrapyramidal, hepatotoxic, hypotensive and photosensitizing effects. Masking of ototoxic symptoms may occur. When used with opioid analgesics,phenothiazines increase CNS & respiratory depression as well as increase orthostatic hypotension and the risk of paralytic ileus and/or urinary retention. Its additive QT prolongation when used with probucol may increase the risk of ventricular tachycardia.
Phenothiazines may reduce effect of the following drugs: Amphetamines (reduced stimulatory effects), apomorphine (decreases emetic response), appetite suppressants, bromocriptine, dopamine (antagonizes peripheral vasoconstriction), ephedrine (decreased pressor effects), levodopa (antiparkinso-nian effects), lithium, metaraminol, mephentermine, methoxamine and phenylephrine. Dosage adjustments may be necessary.

Laboratory Value Alterations
Phenothiazines may interfere with diagnostic test results including producing false-positive result in urine bilirubin tests, blunting response to the gonadorelin tests, and producing false-positives or false-negatives in immunologic urine pregnancy tests. It may cause Q-and T- wave changes in ECG readings and reduces ACTH secretion in metyrapone tests.

PRECAUTIONS

Risk-benefit of prescribing Prochlor should be considered when the following medical problem exists:
Active alcoholism, angina pectoris, blood dyscrasias, breast cancer, cardiovascular disease, glaucoma, patients with hepatic function impairment, Parkinson's disease, patients with peptic ulcer or urinary retention, symptomatic prostatic hypertrophy and those with chronic respiratory disorders especially children.
In children with signs and symptoms suggesting Reye's syndrome, taking prochlorperazine increases the risk of hepatotoxicity.

Effects on Ability to Drive and Use Machine.
Transient drowsiness may occur in some patients during the initial stages of therapy and patients should be advised against the performance of potentially hazardous tasks such as driving a car or operating machinery until the effect has been ascertained.

PREGNANCY AND LACTATION

Neonates exposed to antipsychotic drugs during the third trimester of pregnancy are at risk for extrapyramidal and/or withdrawal symptoms following delivery. There have been reports of agitation, hypertonia, hypotonia, tremor, somnolence, respiratory distress, and feeling disorder in these neonates. These complications have varied in severity; while in some cases symptoms have been self-limited, in other cases neonates have required intensive care unit support and prolonged hospitalisation.
PROCHLOR TABLET 5 should be used during pregnancy only if the potential benefit justifies the potential risk to the foetus.

PRESENTATION

Pack of 10 x 10's, 50 x 10's and 100 x 10's
Not all pack sizes are available locally

STORAGE CONDITIONS AND USER INSTRUCTIONS

Protect from light.
Keep out of reach of children.
Store in a dry place below 30°C.
Jauhi daripada kanak-kanak.
Shelf life: Please refer to outer package.
Do not stop taking this medicine except on your doctors advice

Product Registration Holder:
Duopharma Manufacturing (Bangi) Sdn. Bhd.
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